

Coma - Female (Adult)

Algorithm 1130 | Adult (18 years and older) | Gender at birth: Female

Definition

- UAT-only emergency recognition and first-aid pathway for an unresponsive adult, child, or infant; not approved for real-patient or production use.
- Adult patient.
- Female.

Initial assessment

1. [YES_NO] Is the scene safe to approach, and is the patient unresponsive to voice and gentle touch?
2. [YES_NO] Is the patient breathing normally? Gasping, panting, slow irregular breaths, or uncertainty do not count as normal breathing.
3. [OPEN_TEXT] What is the patient's exact age, how long have they been unresponsive, and what happened immediately beforehand?
4. [YES_NO] Was there trauma, a fall, seizure, choking, drowning, poisoning, overdose, diabetes, pregnancy, or another known medical cause?
5. [YES_NO] Is an AED available, and is there another person who can call Qatar 999 and bring it?

Triage and redirect questions

1. Redirect → Confusion - Delirium

Is the person awake and responsive, with sudden confusion rather than unconsciousness or failure to respond?

A responsive person with sudden confusion belongs in the Confusion - Delirium pathway. Any unresponsive person remains in this emergency pathway.

1. Disposition level 100

Is the adult, child, or infant unresponsive to voice and gentle touch, or not breathing normally? Treat gasping, panting, slow irregular breaths, and uncertainty about normal breathing as a possible cardiac arrest.

Resuscitation Council UK 2025 guidance requires an immediate emergency call for any unresponsive person and CPR when breathing is abnormal. Adults and children with reduced responsiveness who do not meet CPR criteria may be placed laterally, except in trauma or agonal breathing.

Advice

Unresponsive patient â€” call Qatar 999

- Call Qatar 999 immediately, put the phone on speaker, and follow the operator's instructions. If the patient is unresponsive and not breathing normally, start age-appropriate CPR immediately and use an AED as soon as available, following its prompts. If breathing normally and there is no suspected trauma, place the patient in a lateral recovery position and continuously monitor breathing. If trauma or spinal injury is possible, avoid unnecessary movement but keep the airway open as directed by 999; airway and breathing take priority. Do not give food, drink, oral sugar, or medicine to an unresponsive patient and do not leave them alone. Use naloxone or another emergency medicine only if available, appropriate to the suspected cause, and directed by the patient's existing plan, product instructions, a trained responder, or the 999 operator. Do not drive the patient to hospital. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: breathing stops or changes; person starts to wake but remains confused.

First aid

- Call Qatar 999 immediately, put the phone on speaker, and follow the operator's instructions. If the patient is unresponsive and not breathing normally, start age-appropriate CPR immediately and use an AED as soon as available, following its prompts. If breathing normally and there is no suspected trauma, place the patient in a lateral recovery position and continuously monitor breathing. If trauma or spinal injury is possible, avoid unnecessary movement but keep the airway open as directed by 999; airway and breathing take priority. Do not give food, drink, oral sugar, or medicine to an unresponsive patient and do not leave them alone. Use naloxone or another emergency medicine only if available, appropriate to the suspected cause, and directed by the patient's existing plan, product instructions, a trained responder, or the 999 operator. Do not drive the patient to hospital. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- Hamad Medical Corporation, "Life-Threatening Medical Emergency", <https://hamad.qa/EN/Emergency/Pages/Life-ThreateningMedicalEmergency.aspx> (accessed 25 July 2026)
- Resuscitation Council UK, "Adult Basic Life Support Guidelines", 2025, <https://www.resus.org.uk/professional-library/2025-resuscitation-guidelines/adult-basic-life-support-guidelines>
- Resuscitation Council UK, "Paediatric Life Support", 2025, <https://www.resus.org.uk/professional-library/2025-resuscitation-guidelines/paediatric-basic-life-support-guidelines>
- Resuscitation Council UK, "First Aid Guidelines â€” Recovery position", 2025, <https://www.resus.org.uk/professional-library/2025-resuscitation-guidelines/first-aid-guidelines>
- NHS.UK, "Recovery position", <https://www.nhs.uk/tests-and-treatments/first-aid/recovery-position/>

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from the cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect and is not a disposition row. Demographic variants contain only source-supported differences; identical thresholds are

explicitly documented rather than fabricated. Local destination mappings and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

Manifest hash: 3142eb006278e95f539d785ecb80c63d43b8e75d1211e8baae523c48a779420b | Status: BLOCKED_BY_RESEARCH_GAP